

CHOLERA

- If patient has watery diarrhoea (with or without vomiting) and has been in cholera outbreak area in past 5 days, **cholera** likely.
- If possible, isolate patient. Health worker to wear gloves and apron while attending to patient. Disinfect surfaces contaminated with secretions with 70% alcohol or chlorine-based disinfectant.
- Check glucose: if glucose < 3 or > 11 ➔ 17.
- Record each episode of diarrhoea and vomiting and use this to calculate ongoing losses¹ when giving fluid replacement below. Decide on further management according to level of dehydration:

Any of: drowsy/decreased consciousness, confused, difficulty breathing, weak pulse?

Yes

No

≥ 2 of: unable to drink (or drinking poorly), poor skin turgor, sunken eyes?

Yes

No

≥ 2 of: restless, thirsty, dry mouth, pulse ≥ 100?

Yes: **some dehydration** likely

Give **oral rehydration solution (ORS)** 75mL/kg (or at least 2-4L) over 4 hours.

No: dehydration unlikely

Give **ORS**, at least 2L (as much as patient wants) over 4 hours.

Take specimen (see below) and reassess hourly. Either of:

- Signs of dehydration: unable to drink (or drinking poorly), poor skin turgor, sunken eyes, weak pulse (or pulse ≥ 100), not passing urine, unable to walk unaided
- Ongoing diarrhoea or vomiting?

Yes

No: does patient have reliable transport to return if worse?

No

Yes

Manage as severe dehydration and refer urgently:

- Insert 2 large bore IV lines, if possible.
- Ideally give **Ringer's lactate** IV fluid as first choice of fluid replacement. If unavailable, give instead **sodium chloride 0.9% IV**:
 - Give **Ringer's lactate** 30mL/kg IV over first 30 minutes, then **Ringer's lactate** 70mL/kg over next 2.5 hours, while awaiting transfer.
 - Also calculate further IV fluids according to ongoing losses¹ and increase rate of IV fluids to give new total volume of fluid over the 2.5 hours.
 - If transport delayed > 3 hours: continue 5mL/kg/hour IV, increasing rate as needed to keep up with ongoing losses¹.
 - If unable to insert IV lines and unable to drink, insert nasogastric tube (NGT), if able, for giving fluid volumes above. Avoid NGT if vomiting.
- Reassess patient every 15-30 minutes:
 - If no better or worse, discuss with referral centre.
 - If awake, able to drink and pulse < 100, continue IV fluids, but also manage as **some dehydration** (see above).

- Discuss/refer.
- Continue assessing and managing for dehydration hourly (see above).
- If some dehydration, pregnant, ≥ 60 years old or long-term health condition (e.g. HIV, diabetes), give antibiotic:

- Discharge patient into reliable carer's care.
- Advise to return if worse, unable to drink, bloody diarrhoea, fever or becomes drowsy.
- Advise to drink at least 250mL **ORS**² after each stool.
- Advise frequent handwashing with soap and water, before preparing food/after going to toilet. Use only safe/disinfected water for preparing food/drinks/ice.

- Give single dose **ciprofloxacin** 1g orally, especially if pregnant, ≥ 60 years old or long-term health condition (e.g. HIV, diabetes).
- Send stool specimen or rectal swab for MCS and cholera. Ensure correct patient contact and address details appear on lab request form.
 - If laboratory processing delay > 2 hours: place specimen in Cary-Blair transport medium, if available. Place in fridge or cooler box with ice before transport. Avoid freezing.
- Notify as suspected cholera.

¹Calculate ongoing losses: add 10mL/kg of IV fluid/ORS to fluids calculated above for every episode of diarrhoea or vomiting, e.g. if a patient weighing 50kg had 2 episodes of diarrhoea and 1 episode of vomiting during assessment period, add 1500mL of IV/ORS fluid to IV/ORS fluid volumes above [(3 episodes of diarrhoea/vomiting) x (10mL x 50kg)]. ²If possible, give at least 4 sachets of ORS at discharge. Also advise on home ORS solution: add 8 teaspoons of sugar and half a teaspoon of salt to 1L of boiled water. Advise to drink at least 250mL after each stool.